coursera

Anxiety and Related Disorders ✕

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Introduction to Psychological Disorders

Module 3
Opening to Anxiety Disorders

Module 4
Components of Anxiety

Module 5
Disorders that Involve Anxiety Part 1

Module 6
Disorders that Involve Anxiety Part 2

Social Anxiety Disorder
✓ Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Social Anxiety Disorder
Reading • 15 min
✓ Check Your Understanding: Social Anxiety Disorder
Practice Assignment • Grade: 100%

Obsessive-Compulsive Disorder
✓ Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Obsessive-Compulsive Disorder
Reading • 15 min
✓ The Vicious Cycle of OCD
Video • 2 min
○ Check Your Understanding: Obsessive-Compulsive Disorder
Practice Assignment • 4 min

Treatment for Disorders that Involve Anxiety
○ Types of Treatment for Disorders that Involve Anxiety
Reading • 12 min
○ Check Your Understanding: Treatment for Disorders that Involve Anxiety
Practice Assignment • 6 min

Key Takeaways
○ Key Takeaways: Disorders that Involve Anxiety Part 2
Reading • 10 min

Module 6 Quiz
○ Module Quiz: Disorders that Involve Anxiety Part 2
Graded Assignment • 7 min

Module 7
Conclusion

Module 8
Cumulative Quiz

Module 9
Course Resources

Module 10
PsycLearn Essentials APA Student Resources

Types of Treatment for Disorders that Involve Anxiety

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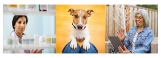


Drug Therapies

Drug treatments and cognitive behavioral therapy have traditionally shown a high degree of effectiveness at decreasing anxiety and treating anxiety disorders. Additionally, clinicians now use technology such as mobile apps and virtual reality therapy to increase the effectiveness of previously established therapeutic techniques. We will describe these various approaches separately, but it is quite common for these interventions to be used in combination to increase the effectiveness of the treatment.

Broadly, two categories of drug therapies have been shown to be effective at decreasing the severity of symptoms in anxiety disorders: **anxiolytic medications** and serotonin-based antidepressants. Anxiolytic medications, also called anti-anxiety medications, attempt to directly lower people's anxiety, often by mildly sedating them. Examples of drugs in this class are benzodiazepines such as Valium and Xanax. These drugs are quite effective at immediately reducing even high levels of anxiety such as a person might experience in a panic attack. However, they are also habit forming and patients can develop a tolerance to the drug and experience withdrawal symptoms if they stop using the drug. For this reason, many researchers and clinicians (e.g., Baldwin et al., 2013) suggest that anxiolytic medications be prescribed for short-term use only.

Selective serotonin reuptake inhibitors (SSRIs) have also been shown to be effective at decreasing anxiety (Gomez et al., 2018). SSRIs are effective because serotonin plays an important role in the activation (and perhaps hyperactivation) of the brain's fear circuit (Duval et al, 2015). Recall that the fear circuit refers to areas of the brain devoted to processing fear- and anxiety-relevant stimuli. SSRIs inhibit the reuptake of serotonin in the neurons' synapses, thus increasing the level of serotonin in the brain. This works to reduce anxiety symptoms by stanching the hyperactivity in the fear circuit. Although SSRIs are, in general, less effective than anxiolytic drugs, they are non-habit forming and therefore more appropriate for long-term use (Gomez et al., 2018).

Much research on obsessive compulsive disorder treatment has concluded that medication should be the first line of treatment. (Kellner, 2010). SSRIs appear to be effective in the treatment of obsessive compulsive disorder. SSRIs increase levels of serotonin in the brain and work by elevating mood (Foa et al., 2005; Kellner, 2010). Serotonin also decreases activity in the frontostriatal circuit, which has been implicated in obsessive compulsive disorder (Kellner, 2010). Recall that overactivity in the frontostriatal circuit fosters intrusive thought and compulsive behavior (Milad & Rauch, 2012).

General anxiety disorder, panic disorder, social anxiety disorder, and obsessive-compulsive disorder are most amenable to treatment with medication. Specific phobias are rarely treated with medication; instead they are best addressed with cognitive behavioral therapy.

Cognitive Behavioral Therapy

Often referred to as the "gold standard" of psychotherapies (David et al., 2018), **cognitive behavioral therapy (CBT)** seeks to teach the patient thought-based and behavior-based coping skills that will help them decrease the anxiety that they are experiencing. For example, a person with generalized anxiety disorder could be taught the cognitive strategy of counterarguing their irrational fears. As another example, someone suffering from panic disorder could be taught a behavioral strategy consisting of controlled breathing and other relaxation techniques to reduce their physiological arousal during an attack.

To give patients practice using anxiety-reduction skills, the skill lessons are often combined with another cognitive behavioral technique—**exposure therapy**. Exposure therapy involves placing patients in situations where they are exposed to stimuli that evoke their anxiety. This exposure helps both by giving patients the opportunity to practice successfully coping with their feared stimuli and by helping them to realize that, despite their anxiety, the negative outcome they feared did not occur.

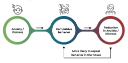
Exposure treatments have been shown to be effective for an array of anxiety disorders including generalized anxiety disorder, panic disorder, specific phobias, and social anxiety disorder (American Psychological Association, n.d.). For example, as a part of a treatment designed to help a patient cope with panic disorder, the patient might be asked to do something that will mimic a symptom of a panic attack. In this case, the therapist would direct the patient to hyperventilate, leading to an increase in their heart rate. This won't pose any danger to the patient, but it does provide them with an opportunity to see that their heart can race without any negative consequences for their health. This type of exposure can also be used to show patients how deep breathing exercises will help them slow down their heart rate on their own.

One type of exposure therapy that has been shown to be especially helpful when treating specific phobias is **systematic desensitization** (Wolpe, 1969). In systematic desensitization, patients are taught anxiety management techniques and then gradually exposed to mild versions of their feared object while practicing relaxation techniques. For example, a person with a spider phobia initially could be shown a picture of a small spider taken from very far away. After successfully reducing their anxiety to this mild stimulus, the patient would view a slightly more intense version of the feared object, such as a close-up picture of a large spider. While using the techniques they've learned to manage the anxiety, the patient will be exposed to more and more intense versions of their feared object. A live spider in a jar across the room could be followed by the same spider in a jar next to the patient and then the spider out of the jar on the couch next to the patient. This process of increasing the intensity of the exposure to the feared object while successfully controlling the fear is repeated until the patient can tolerate being directly exposed to the feared object itself.

CBT is commonly used for treatment of obsessive compulsive disorder (Derisley et al., 2008; O'Kearny et al., 2007). The cognitive aspect of CBT includes psycho-education and cognitive training with the client. Cognitive therapists believe psychological disorders are based, in part, on faulty thought processes (Beck, 1967). Thus, they endeavor to help clients recognize the thought patterns that are contributing to their symptoms (Foa et al., 2005; Libby et al., 2004). For example, a patient whose distress comes from an inflated sense of responsibility for the health of everyone in their city can be asked to reason through this cognitive distortion and recognize the events they actually can control (Barrett et al., 2008).

The behavioral aspect of CBT involves an effort to change behavior patterns, such as the compulsions in obsessive compulsive disorder (Lewin et al., 2011). Compulsive behavior is often based on learned behavior that alleviates anxiety or distress. One behavioral technique used in the treatment of obsessive compulsive disorder is exposure and response prevention—exposure to the feared stimulus and prevention of the ritual response, often while teaching relaxation techniques (Abramowitz et al., 2005; Lindsay et al., 1997). For example, if a patient compulsively avoids stepping on cracks, and must perform some ritual like counting if they do, a therapist might take the patient outside, have them step on cracks and prevent them from engaging in their normal response of counting. This helps the person to see that the anxiety will lessen on its own, especially with cognitive therapy to help patients understand their cognitive distortions. In the image "Negative Reinforcement of Compulsive Behavior" you will see a visual representation of how compulsive behavior can be negatively reinforcing.

Negative Reinforcement of Compulsive Behavior


Anxiety / Distress
Compulsive behavior
Reduction in Anxiety / Distress
More likely to repeat behavior in the future

Note. Anxiety/distress leads to compulsive behavior, which reduces the anxiety/distress, and is negatively reinforced and more likely to be repeated in the future.

Anxiety Treatment in the Digital Age

Technology can be directly integrated with CBT techniques because of their regimented, predictable, skill-based nature. For example, the use of virtual reality exposure therapy (VRET) can allow people to have a direct exposure to their feared object or situation in a safe, controlled environment. Recent studies have demonstrated that patients suffering from specific phobias who were given VRET showed the same amount of real-world symptom improvement as in-person exposure therapy patients (Morina et al., 2015).

The benefits of VRET are not restricted to specific phobias. Anna Felnhofer and her colleagues (2019) showed that exposing people with social anxiety to virtual social stress scenarios, such as ordering a drink from a waiter who subsequently brings the wrong drink, creates both the psychological and physiological anxiety reactions present in the real-world versions of these social stress scenarios. Because VRET is so effective at creating real-world anxieties, it is only a matter of time before therapists will have VRET setups that will allow them to expose patients to complex social scenarios in their office—something that was simply not possible previously.

Modern technology can also be leveraged to help people retrain their focus of attention away from anxiety-producing stimuli. In one example, the **attention bias modification (ABM) task**, patients are shown anxiety-evoking stimuli alongside non-anxiety evoking stimuli and, over repeated trials, are trained to pay more attention to the non-anxiety evoking stimuli. This treatment reduces the power of the anxiety-evoking stimuli to capture their attention, thereby reducing the patient's overall level of anxiety (Mogg et al., 2017). The theoretical foundation that underlies this treatment has been known for decades, but it's only recently that researchers (Dennis & O'Toole, 2014) have created gamified mobile apps that have transformed this technique from one that was only available during a therapy session to one that could be used and practiced by people wherever and whenever they want. Who knew that video games can treat anxiety?!

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